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File No.: EXP202304215

RESOLUTION OF RIGHTS PROCEDURE

Following the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: Through a burofax received on February 1, 2023, Mr. B.B.B. (hereinafter, the representative) on behalf of Mr. A.A.A. (hereinafter, the claimant) requested CENTRO DENTAL ALFAZ DEL PI, S.L. (hereinafter, the respondent) access to a complete copy of his medical history and that it be sent to his email address. This request was also signed by the claimant himself.

The legal representative of the respondent, in a letter dated February 14, 2023, communicated that he had not provided the necessary power of representation, and that the requested documentation was available to the claimant at his office.

By email dated February 17, 2023, sent to the address listed in the aforementioned communication dated February 14, 2023, ***EMAIL.1, the legal representative of the claimant sent a copy of the power of representation.

SECOND: In accordance with Article 65.4 of the LOPDGDD, the aforementioned claim was forwarded to the respondent for analysis and to inform this Agency within one month of the actions taken to comply with the requirements established in data protection regulations.

The respondent indicated that, after receiving a burofax requesting the medical history of the claimant, to which the necessary power of representation was not attached nor a copy of the patient's ID, they responded, via burofax, "(...) noting the lack of such documents and in which, following the provisions of Law 41/2002, of November 14, the basic regulatory law of patient autonomy and rights and obligations in terms of information and clinical documentation, the procedure and requirements for obtaining the requested medical history were communicated."

"On February 17, SANTAMARIA BAEZA ABOGADOS sent an email to which a power of representation (Document 4), an authorization for the sending of the medical history via email (Document 5), and a copy of the ID of Mr. A.A.A. (Document 6) were attached.

Due to the discrepancy presented by the signature contained in the ID compared to the signatures included in the other cited documents, a report was requested from a judicial handwriting expert (Document 7) who determined the insufficiency of the power of representation, an insufficiency based on:

- a) The signatures contained in the documentation submitted show evident discrepancies and divergences with respect to the authentic signature contained in the patient's ID.
- b) There is evidence that the signatures were inserted into the submitted documents using the "copy and paste" procedure, without being able to confirm the personal participation, knowledge, and consent of the patient whose medical history is requested.
- c) The purported power of representation (Document 4), dated January 12, and whose existence was announced on January 31 via the burofax (Document 1) sent by SANTAMARIA BAEZA ABOGADOS, was actually drafted on February 17 (Document 7).

On the same day, February 17, a response was sent via email to SANTAMARIA BAEZA ABOGADOS, explaining the deficiencies observed in the submitted documentation and requesting rectification of such deficiencies (Document 8).

A response received by SANTAMARIA BAEZA ABOGADOS, which on February 20 replied via email (Document 9), refusing to make the requested rectification.

On the same day, February 20, a new email was sent to SANTAMARIA BAEZA ABOGADOS with a further explanation regarding the serious deficiencies observed (Document 10), reiterating the request for rectification, and, failing that, requesting that the patient personally visit the clinic to receive a copy of his medical history.

A response received by SANTAMARIA BAEZA ABOGADOS, which again replied via email on the same day, February 20 (Document 11), reiterating its refusal to rectify.

On the same day, February 20, a response was sent to SANTAMARIA BAEZA ABOGADOS via email (Document 12), explaining that, in the absence of a duly accredited power of representation, the patient must appear in person at the clinic to receive a copy of his medical history.”

THIRD: Regarding the letter sent in response to the information request made by this Agency, the respondent was informed that there were certain aspects that needed clarification.

For this reason, a deadline was given to supplement the documentation initially submitted, providing evidence of the response to the exercise of the right of access.

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FOURTH: In response to the request for information, the responding party “(...) communicates that the response to the request was sent along with all the documentation provided in my previous writing.

Regarding this matter, the communications sent via email to SANTAMARÍA BAEZA ABOGADOS SL are again attached, in which they are requested, three times, to remedy the deficiencies observed in the documentation they provided and that, failing that, it should be the patient who directly contacts the clinic to exercise their right of access, which has never been denied.”

“(…) Thus, and given that the only power of representation provided by SANTAMARÍA BAEZA ABOGADOS SL to the clinic suffers from serious deficiencies that raise doubts about its veracity, in protection of the mandatory confidentiality of the patient's data, the aforementioned law firm has been requested to remedy the deficiencies in the documentation submitted to the clinic and, consequently, to duly certify the representation.”

FIFTH: The result of the transfer procedure indicated in the previous Fact did not allow understanding that the claims of the complaining party were satisfied. Consequently, on June 8, 2023, for the purposes provided in Article 64.1 of the LOPDGGD, the Director of the Spanish Agency for Data Protection agreed to admit the complaint for processing.

The aforementioned agreement granted the responding party a hearing procedure, so that within ten working days they could present the allegations they deemed appropriate, stating, in summary, that “(...) it must be remembered that the Agency to which I am addressing cannot disregard Law 41/2002, of November 14, which regulates the autonomy of the patient and rights and obligations regarding clinical information and documentation, whose Article 7.1 establishes that Every person has the right to have the confidentiality of data concerning their health respected, and that no one may access them without prior authorization protected by law.

Likewise, the second paragraph of the same Article 7 establishes that Health centers will adopt the appropriate measures to guarantee the rights referred to in the previous paragraph, and will develop, when appropriate, the rules and protocolized procedures that guarantee legal access to patient data. In this regard, the health center sent a burofax to SANTAMARÍA BAEZA ABOGADOS SL communicating the procedure to follow in order to claim a patient's clinical file, notifying that, following Article 18 of the aforementioned legal body, a duly accredited power of representation was required. A requirement mandated by law, since, according to Article 17.4 of the aforementioned Law, The custody of such clinical histories shall be the responsibility of the management of the health center. (...)”

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“On the other hand, it should be noted that in the communication sent by SANTAMARÍA BAEZA ABOGADOS SL, it was expressly stated that the legal representative of the clinic could not send said history electronically, so the only way to receive said history, according to SANTAMARÍA BAEZA ABOGADOS SL, was to go personally to the clinic.”

“(…) Finally, this party can only state that, at all times, the option has been given to DON A.A.A. to receive his clinical history by going to the clinic. It should be noted that the law does not require the center to send said documentation electronically, being able to establish, within the powers that the law offers, the procedure for its delivery, which may establish the only possibility of in-person delivery, as most public hospital centers integrated into the National Health System do.”

FOURTH: After examining the document submitted by the responding party, it is transferred to the complaining party, so that, within ten working days, they may formulate the allegations they consider appropriate, not formulating allegations within the established period.

LEGAL GROUNDS

I

Competence

In accordance with the powers granted to each supervisory authority by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and as established in Articles 47, 48.1, and 64.1 of the LOPDGDD, the Director of the Spanish Agency for Data Protection is competent to initiate and resolve this procedure.

Furthermore, Article 63.2 of the LOPDGDD states that "The procedures processed by the Spanish Agency for Data Protection shall be governed by the provisions of Regulation (EU) 2016/679, by this organic law, by the regulatory provisions issued in its development and, as long as they do not contradict them, subsidiarily by the general rules on administrative procedures."

II

Preliminary Issues

In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among controllers and processors about their obligations, as well as addressing complaints filed by an interested party and investigating, as appropriate, the reasons for them.

Correspondingly, Article 31 of the GDPR establishes the obligation of controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In cases where they have designated a data protection officer, Article 39 of the GDPR assigns this officer the function of cooperating with that authority.

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In accordance with this regulation, following the procedure established in Article 65.4 of the LOPDGDD, prior to the admission of the claim for the exercise of the rights regulated in Articles 15 to 22 of the GDPR, which gives rise to this procedure, the claim was forwarded to the party being complained against for them to analyze it, respond to this Agency within one month, and provide evidence of having given the claimant the appropriate response.

The result of this forwarding did not allow for the understanding that the claims of the complaining party were satisfied. Consequently, on June 8, 2023, for the purposes provided in Articles 64.1 and 65 of the LOPDGDD, the claim submitted was admitted for processing. This admission for processing determines the opening of the present procedure for the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, according to which:

“When the procedure refers exclusively to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of Article 65 of this organic law.

In this case, the deadline for resolving the procedure shall be six months from the date on which the admission agreement was notified to the claimant. After this period, the interested party may consider their claim accepted.”

Article 58.2 of the GDPR grants the Spanish Data Protection Agency a series of corrective powers for the purpose of rectifying any infringement of the GDPR, among which is included "ordering the data controller or processor to attend to the requests for the exercise of the rights of the data subject under this Regulation."

III

Rights of individuals regarding personal data protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability are contemplated.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, consideration is given to what is expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of their rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to the requests made no later than within a month.

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unless it can demonstrate that it is unable to identify the interested party, and to express its reasons in case it does not intend to comply with such request. The burden of proof regarding the fulfillment of the duty to respond to the request for the exercise of rights made by the affected party lies with the controller.

The communication addressed to the interested party in connection with their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

IV

Right of Access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the interested party has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data." Regarding the right of access to personal data, according to the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the controller may request the affected party to specify the "data or processing activities to which the request refers." The right will be deemed granted if the controller provides remote access to the data, considering the request fulfilled (although the interested party may request the information referred to in the aspects provided in Article 15 of the GDPR).

The exercise of this right may be considered repetitive if it is exercised more than once within a period of six months, unless there is a legitimate reason for doing so. On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that entails a disproportionate cost, which must be borne by the affected party.

Like the other rights of the interested party, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them and being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decision-making, including profiling, and information about transfers of personal data to a third country or to an international organization. The

possibility of obtaining a copy of the personal data being processed shall not adversely affect the rights and freedoms of others.

The right of access in relation to medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, which is the basic regulatory law of the...

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Patient Autonomy and Rights and Obligations Regarding Information and Clinical Documentation (hereinafter LAP), the literal text expresses:

- "1. The patient has the right to access, with the reservations indicated in section 3 of this article, the documentation of the clinical history and to obtain a copy of the data contained therein. Healthcare centers will regulate the procedure that guarantees the observance of these rights.
2. The patient's right to access the clinical history may also be exercised by duly accredited representation.
3. The patient's right to access the documentation of the clinical history cannot be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the rights of the professionals involved in its preparation, who may oppose the right of access based on the confidentiality of their subjective notes.
4. Healthcare centers and individual practitioners will only facilitate access to the clinical history of deceased patients to individuals linked to them, for family reasons or otherwise, unless the deceased expressly prohibited it and this is evidenced. In any case, access by a third party to the clinical history motivated by a risk to their health will be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, will not be provided."

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the clinical history:

- "1. The clinical history will incorporate the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in primary care and specialized care.
2. The main purpose of the clinical history is to facilitate healthcare, documenting all those data that, under medical criteria, allow for the truthful and updated knowledge of the health status.

The minimum content of the clinical history will be as follows:

- a) Documentation related to the clinical-statistical sheet.
- b) Admission authorization.
- c) Emergency report.
- d) Anamnesis and physical examination.
- e) Evolution.
- f) Medical orders.
- g) Consultation sheet.
- h) Reports of complementary examinations.
- i) Informed consent.
- j) Anesthesia report.
- k) Operating room report or delivery record.
- l) Pathological anatomy report.
- m) Nursing care evolution and planning.
- n) Nursing therapeutic application.
- ñ) Constants chart.
- o) Clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) will only be required in the completion of the clinical history when it concerns hospitalization processes or as stipulated.

3. The completion of the clinical history, in aspects related to direct patient care, will be the responsibility of the professionals involved in it.

4. The clinical history will be maintained with criteria of unity and integration, in each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the professionals of the data of a particular patient in each care process."

Regarding the preservation of the clinical history, Article 17 of the LPA, in its points 1 and 5, states that:

"1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original support, for the appropriate assistance to the patient for the time suitable for each case and, at a minimum, five years from the date of discharge from each care process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate."

V

Conclusion

The rights procedure is instructed as a consequence of the denial of any of the rights regulated in the data protection regulations. Therefore, in this case, only the aspects related to the access request of the claiming party will be analyzed and evaluated, excluding the rest of the issues raised, such as the validity or not of the claimant's signature on the representation document.

Article 11 of the GDPR states that

"Article 11 Processing that does not require identification

1. If the purposes for which a controller processes personal data do not require or no longer require the identification of a data subject by the controller, the controller shall not be obliged to maintain, obtain, or process additional information for the sole purpose of identifying the data subject in order to comply with this Regulation."

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2. When, in the cases referred to in paragraph 1 of this article, the data controller is able to demonstrate that they are unable to identify the data subject, they shall inform them accordingly, if possible. In such cases, Articles 15 to 20 shall not apply, except when the data subject, for the purposes of exercising their rights under those articles, provides additional information that allows for their identification."

And Article 12, paragraphs 2 and 6 of the same legal text:

"Article 12

2. The data controller shall facilitate the exercise of the data subject's rights under Articles 15 to 22. In the cases referred to in Article 11, paragraph 2, the data controller shall not refuse to act at the request of the data subject in order to exercise their rights under Articles 15 to 22, unless they can demonstrate that they are unable to identify the data subject.

6. Without prejudice to the provisions of Article 11, when the data controller has reasonable doubts regarding the identity of the natural person making the request referred to in Articles 15 to 21, they may request the additional information necessary to confirm the identity of the data subject." (the underlining is ours)

The Guidelines 01/2022, from the European Data Protection Board, on the rights of data subjects - Right of access, in its version 2.0, adopted on March 28, 2023, regulate in their section 3.4 the access requests made by third parties:

"80.

Although the right of access is generally exercised by data subjects regarding themselves, it is possible for a third party to make a request on behalf of the data subject. This may apply, among others, to acting through a representative or legal guardian on behalf of minors, as well as to acting through other entities via online portals. In some circumstances, the identity of the person authorized

to exercise the right of access, as well as the authorization to act on behalf of the data subject, may require verification, when appropriate and provided (see section 3.3). It should be noted that providing personal data to a person who does not have the right to access it may constitute a violation of personal data.

81.

In doing so, national laws governing legal representation (for example, powers of representation) must be taken into account, which may impose specific requirements to demonstrate the authorization to submit a request on behalf of the data subject, as the GDPR does not regulate this issue. In accordance with the principle of accountability, as well as with the other principles of data protection, data controllers may demonstrate the existence of the relevant authorization to submit a request on behalf of the data subject and receive the requested information, unless national legislation differs (for example, national legislation contains specific rules regarding the reliability of lawyers), leaving it to the data controller to verify the identity of the..."

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attorney-in-fact (for example, in the case of lawyers verifying registration with the bar). Therefore, it is recommended to gather the appropriate documentation in this regard, in relation to the general rules previously indicated concerning the confirmation of the identity of a natural person making a request, and if the data controller has reasonable doubts about the identity of a person acting on behalf of the interested party, they will request additional information to confirm the identity of that person." (The underlining is ours)

Regarding the personal nature of the rights of individuals in matters of personal data protection and the powers of attorney for third parties to exercise them on behalf of others, reference is made to the Judgment of the National Court of May 20, 2010, Case Number: 106/2009, which states that "FIFTH.-

(...) Thus, the action of the Diana clinic, requiring an express power of attorney for the exercise of the specific right that the complainant intended to exercise, is in accordance with the personal nature of such rights and with what is established in Instruction 1/1998 of the Data Protection Agency, which is mandatory compliance, and which in its First Rule, section 3, establishes that the exercise of rights must be carried out by means of a request addressed to the data controller that will contain the name, surname of the interested party and a photocopy of the interested party's ID card "and, in cases where it is exceptionally admitted, of the person who represents them as well as a document proving such representation," adding in section 4 "In the event that the request does not meet the specific requirements in section three, the data controller must request the rectification of the same."

However, the complainant did not grant such a specific and concrete power of attorney as it is not a general power for litigation, and therefore, the action of the Diana clinic cannot be deemed formalistic but rather respectful of the rights of the affected party, who was required to rectify such lack of individual and specific power of attorney, thus making the unequivocal will of the complainant to exercise such right indisputable. Therefore, the clinic did not hinder the exercise of the rights of access, rectification, or cancellation of the appellant, and the Agency's decision on this matter is in accordance with the law. (...)"

In the present procedure, from the examination of the documentation provided, it has been established that the claiming party, through a voluntary representative, submitted a request for access to their medical history to the responding party, and that the latter, in light of the doubts raised by the representation document presented, requested that it be rectified or, failing that, that the claiming party personally come to collect the requested medical history, after identification.

Consequently, given that the right of access to the medical history is a personal right, given the reasonable doubts of the data controller regarding the veracity of the representation document and that it offered alternative means to obtain a copy of the requested medical history, it cannot be considered that the responding party acted in a formalistic manner, but rather in a manner respectful of

the rights of the applicant. Therefore, it is appropriate to dismiss the claim that led to this procedure.

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In view of the cited provisions and others of general application, the Director of the Spanish Data Protection Agency RESOLVES:

FIRST: TO DISMISS the claim made by Mr. A.A.A. against CENTRO DENTAL ALFAZ DEL PI, S.L.

SECOND: TO NOTIFY this resolution to Mr. A.A.A. and to CENTRO DENTAL ALFAZ DEL PI, S.L.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Data Protection Agency within one month from the day following the notification of this resolution or directly file an administrative contentious appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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